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ICU

YG CG1005 DEC 2020

PREVENTION AND TREATMENT OF CONSTIPATION IN CRITICAL CARE

REVIEW BY (DEC 2023)

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Key Points

- GI motility issues are common in critically ill patients
- Constipation and its implications receive relatively little attention when compared to diarrhoea
- Constipation can have serious implications for patients in Critical Care

INTRODUCTION

Constipation is documented to affect anything from 5% – 90.5% of ITU patients. It tends to be overlooked within the grand scheme of problems of the Critically Ill. However, constipation in critically ill patients can have significant consequences for the patients and serious implications for their ongoing management.

MODIFIABLE RISK FACTORS

Delayed enteral nutrition	Reduced mobility	Sedation / opiate usage
Anticholinergics	Calcium channel blockers	Paralysing infusions
Electrolyte disturbances	Dehydration	Low BP / Hypoxia

NON MODIFIABLE RISK FACTORS

Abdominal surgery	Severe illness	Spinal Paralysis
Prior alcohol or cannabis	Nicotine withdrawal	

CONSEQUENCES

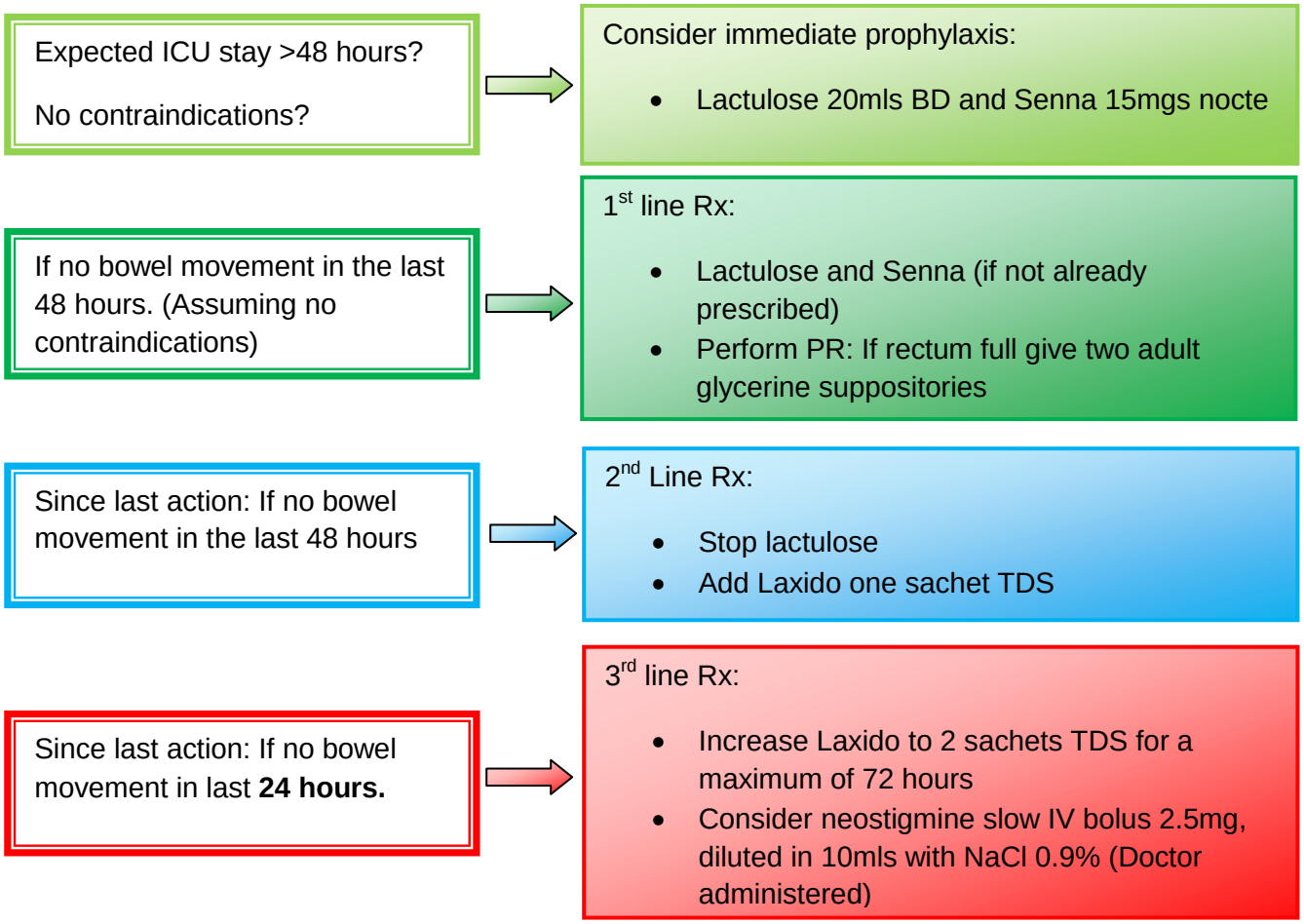
Abdominal distention	Vomiting and aspiration	Increased intra-abdominal pressure
Reduced nutritional intake	Hyperproliferation of bacteria leading to translocation	
Bowel obstruction	Bowel perforation	Gut ischaemia

The implications of the above for ICU patients include: Increased length of stay, Slow ventilatory wean, agitation and delirium and increased mortality.

For patients with a predicted stay of more than 48 hours there is a case for prescribing first line laxative treatment, unless known to have diarrhoea.*

(* ensure that diarrhoea due to overflow from constipation is excluded)

Pathway: Patients <u>without</u> known or suspected bowel pathology



** For patients with possible/known bowel pathology or recent surgery consult surgical team **

(Taken from North Wales Critical Care Network guidelines for the prevention and treatment of constipation in Critical Care)

http://www.wales.nhs.uk/sites3/documents/753/Guideline%20for%20the%20Prevention%20and%20Treatment%20of%20Constipation%20FINAL_APPROVED.pdf